

FORM-ADJ-15B**LOPE Blinded Case Narrative Template (LOPE-Nigeria)**

ACRN Independent OAC · PROTECT-Africa (EOPE) & LOPE-Nigeria (LOPE) · Version 1.0

Purpose. Standardized, de-identified clinical narrative forming the body of the blinded LOPE case package (SOP-ADJ-001 §5.2; OAC Charter §6). Completed by the Adjudication Coordinator; released only after the FORM-ADJ-01 blinding check has passed; read alongside FORM-ADJ-03 (LOPE structured adjudication form). Cross-referenced to LOPE-Nigeria protocol ACRN-202503 v1.1 (eligibility & cohorts §4; Schedule of Activities Table 2; composite endpoints).

BLINDING NOTICE — Do NOT record any biomarker or point-of-care result (sFlt-1/PIGF, sEng, Cerca Red, Roche Elecsys® or any POC value, category or derived risk value), site name, provider name or other direct identifier. Use the participant study ID only. Both protocols are non-interventional and these results are withheld from care and from adjudicators until database lock.

1 Case identification

Study	LOPE-Nigeria (LOPE)
Case ID (participant study ID)	
Site code (blinded)	
Cohort	
Prediction window assessed	
Narrative prepared by (Coordinator)	
Date prepared / version	

Cohort: ☐ Suspected LOPE (main) ☐ Normotensive reference cohort

Reference-cohort rule — For a normotensive reference-cohort case, confirm the pregnancy remained outcome-free (no PE / eclampsia / HELLP) within the assessment window before classifying. Reference cases establish gestational-age-specific biomarker ranges and must not be intermingled with the suspected-LOPE cohort.

2 Comprehensive pregnancy & obstetric history

Maternal age (years)	
Date of enrolment	
Gestational age at enrolment (35+1 to 36+6 wk; ≤10% ≥37 wk)	
Gravidity / parity / living children	
Previous pregnancies & outcomes	
Previous miscarriage / stillbirth / neonatal death	

Relevant prior medical history (tick all that apply)

☐ Chronic hypertension
 ☐ Gestational hypertension / PIH
 ☐ Prior preeclampsia
 ☐ Prior eclampsia
☐ Prior HELLP
 ☐ Proteinuria
 ☐ Renal disease
 ☐ Diabetes
☐ Autoimmune (SLE / APS)
 ☐ Thrombophilia
 ☐ None of the above

Gestational-age dating (protocol requires USS ≤ 22 weeks)

Ultrasound before 22 weeks performed? ☐ Yes ☐ No

USS date & gestational age confirmed	
LMP / dating basis	

3 Eligibility & baseline phenotype

Suspected-LOPE inclusion: GA 35+1–36+6 wk (USS ≤22 wk), singleton, clinical suspicion of PE by ≥1 criterion below, age ≥18 y. Reference cohort: singleton, normal BP and no other health issues, GA ≥35 wk, age ≥18 y.

Qualifying suspicion criterion (suspected-LOPE; tick all that applied)

- ☐ New-onset elevated BP (SBP ≥130 / DBP ≥80) ☐ Aggravation of pre-existing hypertension
☐ New-onset proteinuria ☐ Aggravation of pre-existing proteinuria
☐ Other suspicion (symptoms / low platelets / ↑transaminases / suspected IUGR / abnormal uterine Doppler)
- Eligibility pathway:** ☐ Objective criterion-based ☐ Symptom-only ☐ Mixed ☐ Reference (normotensive)

Exact qualifying evidence (values/findings that satisfied the criterion)

Baseline hypertension phenotype

Classification rule — A participant on antihypertensive treatment for documented hypertension is classified as hypertensive irrespective of current BP. Do not record such a participant as “normotensive” on the basis of a normal Visit 1 BP. Note exclusions: proteinuria ≥2+ with elevated BP or on antihypertensives, confirmed HELLP, MgSO₄/eclampsia at enrolment.

- Baseline status:** ☐ Truly normotensive ☐ Treated gestational HTN / PIH
- ☐ Treated chronic hypertension ☐ Worsening chronic hypertension ☐ Superimposed-PE risk

Screening BP & repeat BP (≥6 h apart, within 1 wk)	
Antihypertensive medication history (agent, dose, start)	
Proteinuria & laboratory values at screening	
IUGR / Doppler / fetal findings used for eligibility	

4 Serial study assessments

Follow the LOPE Schedule of Activities (Table 2). Present chronologically; show the series across visits. Add unscheduled / early-termination visits as needed.

Screening (<24 h before V1)

Date / GA at visit	
BP (systolic/diastolic, mmHg) · pulse · other vitals	
Proteinuria (dipstick / spot P:C / 24-h)	
Labs — CBC, platelets, AST, ALT, LDH, creatinine	
Serum / urine biomarker sample collected? (result withheld — blinded)	

PE status this visit: ☐ No PE ☐ Suspected PE ☐ PE ☐ Severe PE ☐ Eclampsia ☐ HELLP

Care escalation: ☐ None ☐ Admission ☐ Referral ☐ MgSO₄ started ☐ Delivery expedited

Symptoms · maternal status · fetal/neonatal status · any AE/SAE (blinding-compliant)

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V1 — Enrolment (35+1–36+6 wk)

Date / GA at visit	
BP (systolic/diastolic, mmHg) · pulse · other vitals	
Proteinuria (dipstick / spot P:C / 24-h)	
Labs — CBC, platelets, AST, ALT, LDH, creatinine	
Serum / urine biomarker sample collected? (result withheld — blinded)	

PE status this visit: ☐ No PE ☐ Suspected PE ☐ PE ☐ Severe PE ☐ Eclampsia ☐ HELLP

Care escalation: ☐ None ☐ Admission ☐ Referral ☐ MgSO₄ started ☐ Delivery expedited

Symptoms · maternal status · fetal/neonatal status · any AE/SAE (blinding-compliant)

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V2 (1 wk ±1 d after V1)

Date / GA at visit	
BP (systolic/diastolic, mmHg) · pulse · other vitals	
Proteinuria (dipstick / spot P:C / 24-h)	
Labs — CBC, platelets, AST, ALT, LDH, creatinine	
Serum / urine biomarker sample collected? (result withheld — blinded)	

PE status this visit: ☐ No PE ☐ Suspected PE ☐ PE ☐ Severe PE ☐ Eclampsia ☐ HELLP

Care escalation: ☐ None ☐ Admission ☐ Referral ☐ MgSO₄ started ☐ Delivery expedited

Symptoms · maternal status · fetal/neonatal status · any AE/SAE (blinding-compliant)

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V3 (2 wk ±1 d after V1)

Date / GA at visit	
BP (systolic/diastolic, mmHg) · pulse · other vitals	
Proteinuria (dipstick / spot P:C / 24-h)	
Labs — CBC, platelets, AST, ALT, LDH, creatinine	
Serum / urine biomarker sample collected? (result withheld — blinded)	

PE status this visit: ☐ No PE ☐ Suspected PE ☐ PE ☐ Severe PE ☐ Eclampsia ☐ HELLP

Care escalation: ☐ None ☐ Admission ☐ Referral ☐ MgSO₄ started ☐ Delivery expedited

Symptoms · maternal status · fetal/neonatal status · any AE/SAE (blinding-compliant)

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V4 (4 wk ±1 d after V1, if undelivered)

Date / GA at visit	
BP (systolic/diastolic, mmHg) · pulse · other vitals	
Proteinuria (dipstick / spot P:C / 24-h)	
Labs — CBC, platelets, AST, ALT, LDH, creatinine	
Serum / urine biomarker sample collected? (result withheld — blinded)	

PE status this visit: ☐ No PE ☐ Suspected PE ☐ PE ☐ Severe PE ☐ Eclampsia ☐ HELLP

Care escalation: ☐ None ☐ Admission ☐ Referral ☐ MgSO₄ started ☐ Delivery expedited

Symptoms · maternal status · fetal/neonatal status · any AE/SAE (blinding-compliant)

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V5 — Delivery

Date / GA at visit	
BP (systolic/diastolic, mmHg) · pulse · other vitals	
Proteinuria (dipstick / spot P:C / 24-h)	
Labs — CBC, platelets, AST, ALT, LDH, creatinine	
Serum / urine biomarker sample collected? (result withheld — blinded)	

PE status this visit: ☐ No PE ☐ Suspected PE ☐ PE ☐ Severe PE ☐ Eclampsia ☐ HELLP

Care escalation: ☐ None ☐ Admission ☐ Referral ☐ MgSO₄ started ☐ Delivery expedited

Symptoms · maternal status · fetal/neonatal status · any AE/SAE (blinding-compliant)

V6 / EOS (4 wk postpartum ±7 d)

Date / GA at visit	
BP (systolic/diastolic, mmHg) · pulse · other vitals	
Proteinuria (dipstick / spot P:C / 24-h)	
Labs — CBC, platelets, AST, ALT, LDH, creatinine	
Serum / urine biomarker sample collected? (result withheld — blinded)	

PE status this visit: ☐ No PE ☐ Suspected PE ☐ PE ☐ Severe PE ☐ Eclampsia ☐ HELLP

Care escalation: ☐ None ☐ Admission ☐ Referral ☐ MgSO₄ started ☐ Delivery expedited

Symptoms · maternal status · fetal/neonatal status · any AE/SAE (blinding-compliant)

Unscheduled / ET visit

Date / GA at visit	
BP (systolic/diastolic, mmHg) · pulse · other vitals	
Proteinuria (dipstick / spot P:C / 24-h)	
Labs — CBC, platelets, AST, ALT, LDH, creatinine	
Serum / urine biomarker sample collected? (result withheld — blinded)	

PE status this visit: ☐ No PE ☐ Suspected PE ☐ PE ☐ Severe PE ☐ Eclampsia ☐ HELLP

Care escalation: ☐ None ☐ Admission ☐ Referral ☐ MgSO₄ started ☐ Delivery expedited

Symptoms · maternal status · fetal/neonatal status · any AE/SAE (blinding-compliant)

5 Serial summary matrix (quick reference)

Parameter	Screen	V1	V2	V3	V4	V5 Del	V6/EOS
GA (wk+d)							
BP (S/D)							
Proteinuria							
Platelets							
AST / ALT							
LDH							
Creatinine							
PE status							

6 Maternal endpoint narrative

Classify each separately; do not collapse neonatal outcomes into the maternal endpoint unless a composite is explicitly requested.

Protocol composite maternal adverse outcome (within 14 days): ICU admission, HELLP, eclampsia, maternal death.

Primary maternal classification: ☐ No PE ☐ PE ☐ Severe PE ☐ Eclampsia ☐ HELLP ☐ Superimposed PE

Date / gestational age of diagnosis	
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Maternal composite (within 14 days of Visit 1; tick all; state timing)

☐ ICU admission ☐ HELLP ☐ Eclampsia ☐ Maternal death

☐ Pulmonary edema ☐ Acute renal failure ☐ Stroke / cerebral hemorrhage ☐ DIC ☐ None

Detail & timing of maternal adverse outcome(s)

7 Fetal / neonatal endpoint narrative

Protocol neonatal composite: preterm delivery <37 wk, fetal growth restriction, NICU admission, stillbirth or neonatal death.

☐ Preterm birth <37 wk ☐ SGA / FGR ☐ NICU admission ☐ Stillbirth / neonatal death

☐ RDS ☐ TTN ☐ IVH ☐ NEC ☐ Placental abruption ☐ None

Gestational age at delivery	
Fetal growth / Doppler findings	
Birth weight (g) / centile	
Neonatal admission course	
Outcome & timing	

8 Maternal clinical course (integrated narrative)

Chronological narrative: symptom progression, BP progression, lab progression, diagnosis timing, admission/escalation/treatment, maternal outcome (blinding-compliant)

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9 Concomitant medications & exposures

Medication / exposure	Indication	Start date	Stop date	Temporal relation to outcome

Categories present: ☐ Antihypertensives ☐ Aspirin ☐ Iron / folate ☐ Antibiotics ☐ Supplements / OTC

10 Protocol deviations / data-quality flags

- ☐ Missing / contradictory source data ☐ Source-to-eCRF inconsistency
- ☐ Missing delivery labs ☐ Incomplete postpartum follow-up (6-wk window) ☐ Eligibility concern
- ☐ Timing discrepancy ☐ Missing USS <22 wk ☐ Antihypertensive / HTN misclassification
- ☐ Reference-cohort not confirmed outcome-free ☐ Cohort intermingling

Detail of each flag (blinding-compliant)

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11 Source reconciliation table

Data item	Source document value	eCRF value	Concordant?	Resolution / query ID

12 Missing-data tracker

Field / visit	Status	Query ID	Date raised	Date resolved	Impact on adjudicability

Mark unavailable fields as [To be confirmed], [Pending] or [Not available]. Incomplete follow-up is not “no endpoint.”

13 Panel-ready summary (1-page brief)

What happened (concise clinical synopsis)

Is the endpoint met? Which endpoint(s), and which remain uncertain?

Was the participant correctly classified / cohort correctly assigned at enrolment?

Does this case suggest a broader site or protocol issue?

14 Adjudication question list for the panel

#	Question for the panel	Domain
1		
2		
3		

Role	Name	Signature	Date
Narrative prepared by (Coordinator)			
QA blinding counter-check			